

Children are not small adults.



The panel's own sentence says children go through continuous physiologic and anatomic changes, which is why a model fitted to adults is not the model a child needs.

AJR EXPERT PANEL REVIEW 10.2214/AJR.26.35523

Artificial intelligence (AI) applications have transformed radiology, yet pediatric medical imaging remains substantially underrepresented in AI development, validation, regulation, and implementation.

PEDIATRIC ARTIFICIAL INTELLIGENCE IN RADIOLOGY

The panel named where the gap is.

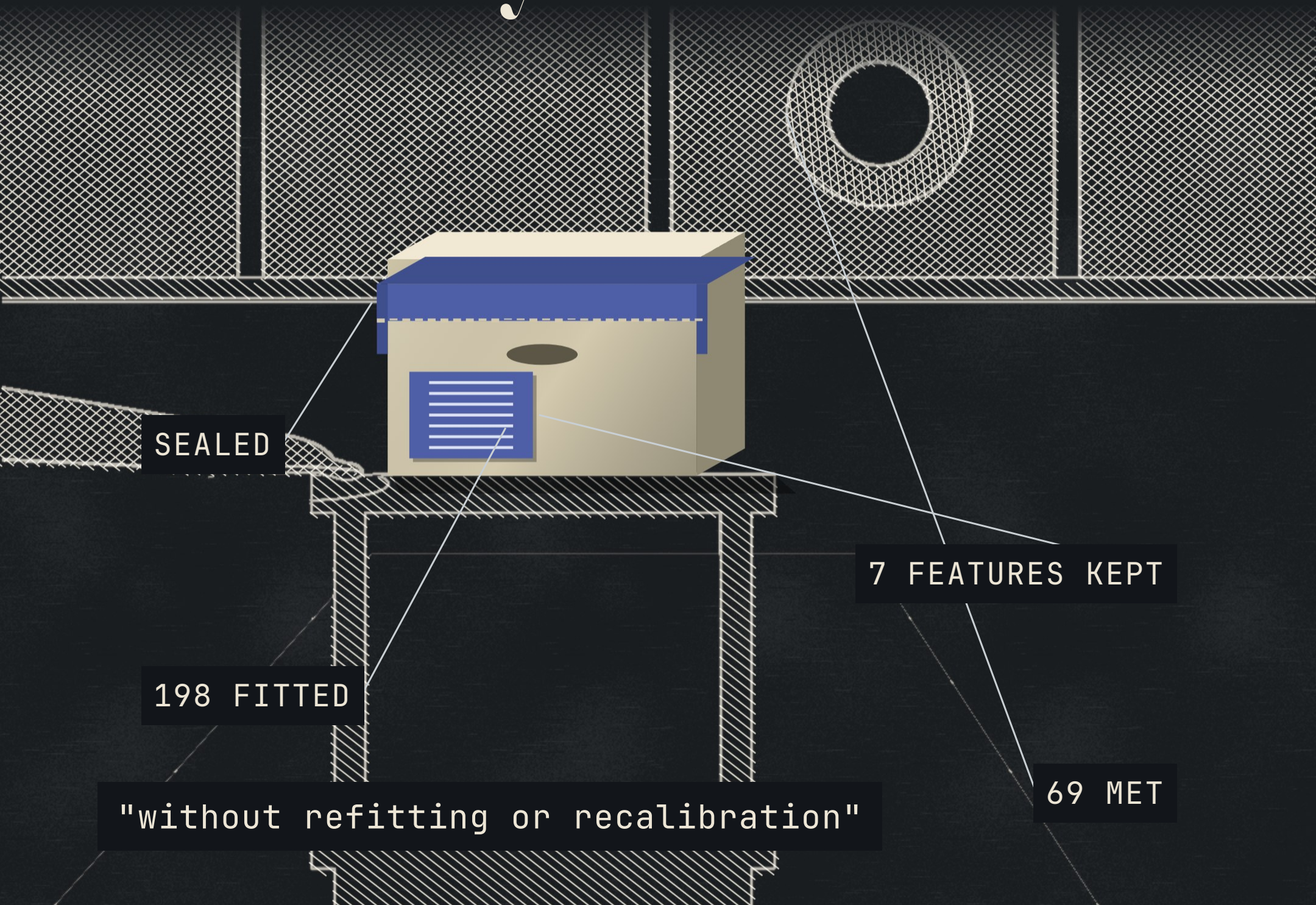
Off label, in the panel's own words.



"off-label use of adult-trained AI models"

The review names the practice among its ethical and regulatory concerns for children, beside consent for secondary data use and the need for postdeployment surveillance.

They sealed the model before they tested it.



The second paper fitted its model on one brain metastasis cohort, kept 7 features, and then ran the fixed model on 69 patients it had never met.

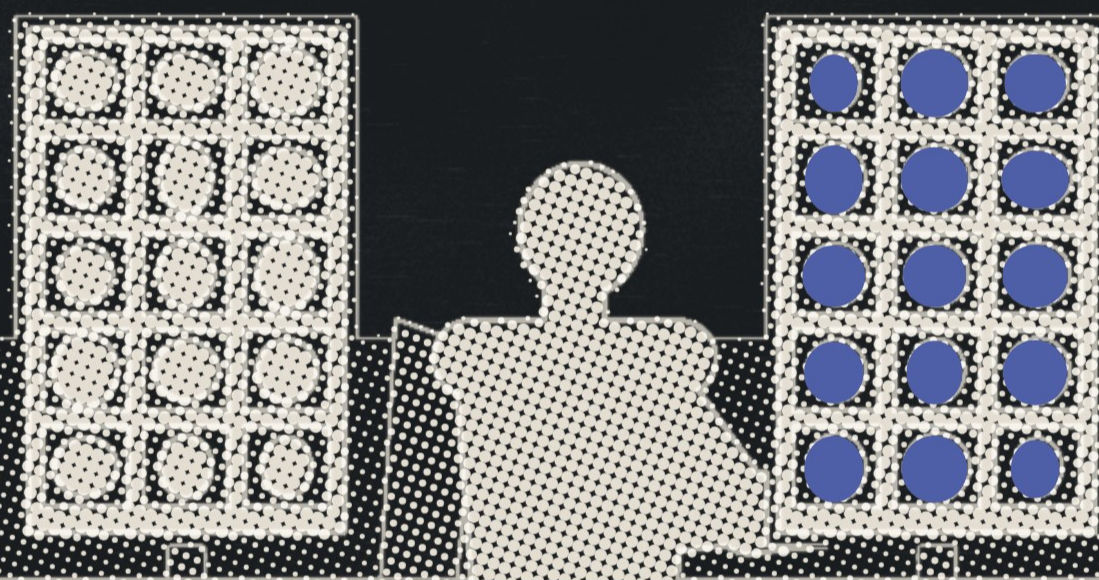
Fitted to 198. Tested on 69.

The model was built on a public cohort. It then ran on an independent cohort without refitting.



It did worse on the patients it had never seen.

"limited standalone discrimination"

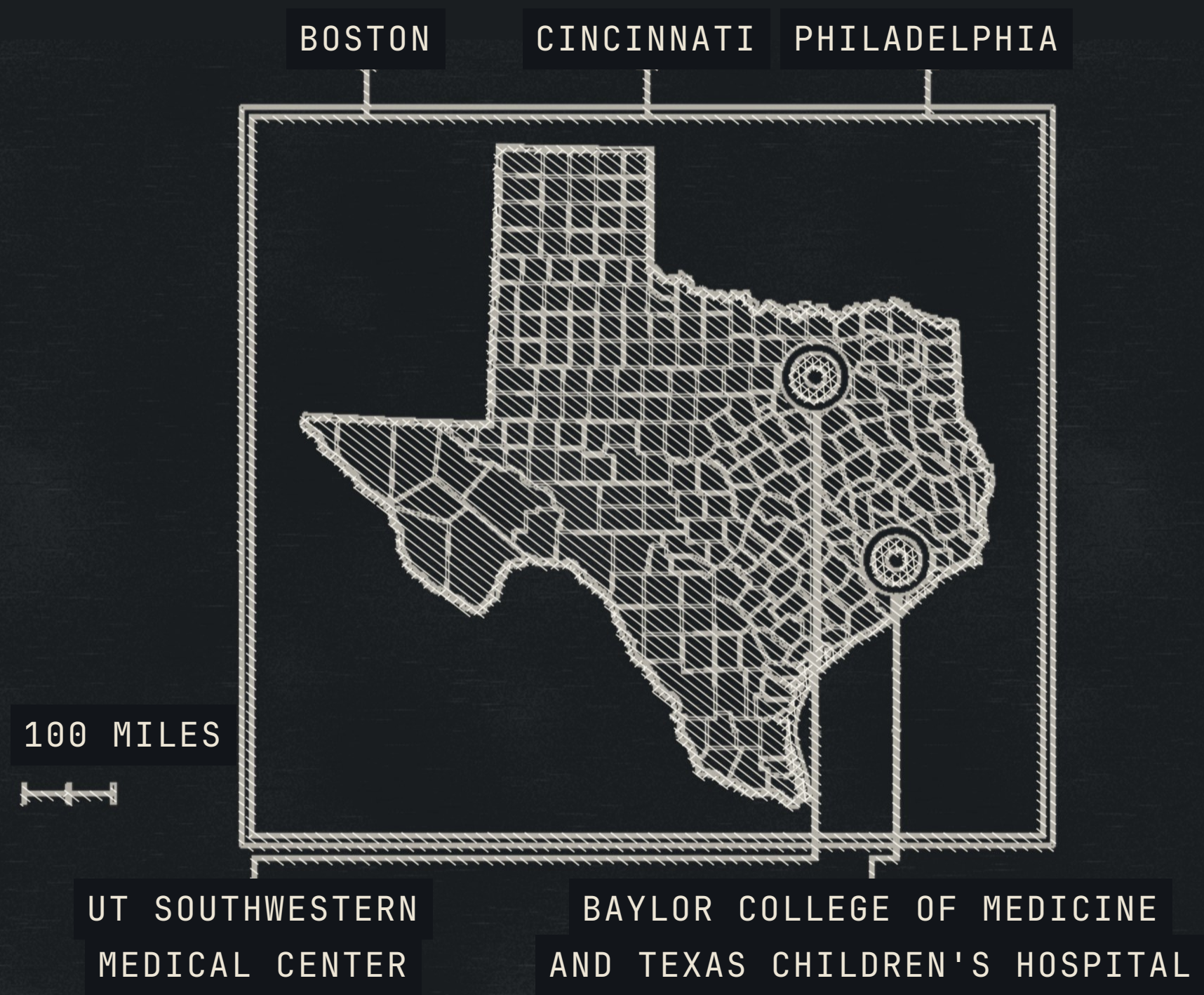


It scored 0.615 on the brain metastasis cohort it was built on and 0.574 on the cohort it had not met. The paper's own intervals are 0.543 to 0.687 and 0.491 to 0.658, and its authors ask for cautious use.

Neither record puts a figure on this.

The review's record names insufficient external validation and carries no measurement of how far outside the training data a child sits.

Two of the panel's departments are Texan.



The two here are at Baylor College of Medicine with Texas Children's Hospital in Houston, and at UT Southwestern Medical Center in Dallas. The three leaving the sheet are the children's hospitals of Boston, Cincinnati and Philadelphia.

WHAT A TEXAN CAN DO
WHERE THE ANSWER IS PUBLISHED

09 / 09

Ask what it was validated on.



A parent can ask at the desk what population each imaging tool was validated on. The Food and Drug Administration's device list carries the word pediatric in two device names and nowhere in its own text. The panel wants pediatric infrastructure, not adult tools reused.